

The Sculpted Face

Protocol.

This is the home routine the studio at FaceSculpt builds every workout around. Twelve chapters, ten to fifteen minutes a day, every clinical claim cited. Strong evidence is named as strong. Weaker evidence is named as weaker. The studio uses this same reference list when patients ask, and updates the document as new trials are published.

The structure is deliberate. Chapters one and two cover the anatomy and the evidence map, so you understand what is happening before you intervene. Chapters three and four are the morning and evening routines, minute by minute. Chapters five through eight cover technique and tools, in the order you would learn them. Chapters nine and ten cover the inputs that show on the face from outside the bathroom. Chapter eleven is honest about where home work ends. Chapter twelve teaches the skill most people skip, which is how to know whether any of it is working.

Read it once, in order. Then keep it for reference.

Length	Daily	Edition	Reading time
Twelve chapters	10 to 15 minutes	2026, version one	25 minutes

How the face *actually* changes.

A face does not age in one place. It ages across four layers, on four different timelines, in response to different inputs. Knowing which layer you are working on tells you which intervention will help, and which is wasted effort.

The four layers, top to bottom

Skin

The outer layer. Loses dermal collagen at roughly one per cent per year after the age of twenty-five.¹ The collagen that remains becomes more cross-linked and less resilient. Fine lines, dryness, and pigment changes happen here. This is the layer with the most evidence behind every intervention. Topical retinoids, vitamin C, sunscreen, and light therapy all work at this level, and the effect compounds over decades.

Fat

Sits in defined compartments under the skin, mapped in detail by Rohrich and Pessa.² With age, the upper-face compartments deflate (temple, lateral cheek), and the lower-face compartments accumulate or shift down (jowl, jaw line). This is the layer where injectables and surgical procedures work. Topicals do nothing here. Manual work does nothing here either. Genetics, weight history and gravity drive most of this.

Muscle

Forty-three named muscles in the face, attached differently from body muscles. Some atrophy with disuse (zygomaticus, levator anguli oris). Some shorten with chronic tension (masseter, corrugator). Lift lines are determined by the tone, position and balance of these muscles. This is the layer microcurrent and EMS work on, and the layer that responds to consistent training. The change is slower than people expect at week four, faster than people expect at week twelve.

Bone

The structural foundation. Resorbs slowly from the late thirties onward, particularly around the eye socket, the mid-face, and the jaw line.³ Shifts the face shape over decades. Nothing topical, manual, or electrical changes this layer. Cosmetic procedures can compensate for bone loss, they do not reverse it.

The window for home work

Two of the four layers are reachable from your bathroom. Skin through topicals and light. Muscle through manual technique and microcurrent. That is the realistic ceiling for home work, and it is a real ceiling. The other two layers (fat and bone) are out of reach without procedures, and most adults would not benefit from procedural intervention in either before their late forties.

One more thing about timing

The earlier you start, the bigger the compound effect. The single highest-yield decision is to begin daily sunscreen and a low-strength retinoid at twenty-eight rather than at forty-eight. Most of the published evidence is on subjects already showing signs of photoaging, which means the recorded results undersell the prevention case.

IN THE STUDIO

One workout, both reachable layers.

Sculpt Signature works the skin and muscle layers in the same forty-minute session. Lymphatic clearance, manual sculpt across the lift lines, microcurrent in held positions, recovery. Photographed and assessed against the file at session one.

Book on +44 20 3951 3429 or in person at 10 Portman Square, W1H 6AZ. £110 for forty minutes.

What works, what doesn't, and what we don't know.

Most beauty advice does not state the strength of the evidence behind a claim. This is the table the studio uses internally. Strong means multiple randomised controlled trials. Moderate means smaller trials, or emerging evidence. Limited means we do not yet know.

INTERVENTION	EVIDENCE	WHAT IT DOES
Daily broad-spectrum SPF	STRONG	Single most evidence-backed anti-ageing intervention. Slows photoaging, prevents skin cancer. ⁴
Topical retinoid (tretinoin, retinol)	STRONG	Reduces wrinkles, evens pigment, improves texture. Gold standard across multiple meta-analyses. ^{5,6}
Vitamin C, L-ascorbic acid 10 to 20 per cent	STRONG	Antioxidant protection, collagen synthesis support, evens pigment over time. ⁷
LED light, 633 nm and 830 nm	STRONG	26 to 36 per cent wrinkle reduction in split-face RCTs. Increased collagen density on ultrasound. ^{8,9}
Niacinamide 5 per cent	STRONG	Improves barrier function, reduces redness, evens pigment. ¹⁰
Sleep duration and quality	STRONG	Poor sleepers show measurable signs of accelerated intrinsic ageing. ¹⁶
Microcurrent on facial muscles	MODERATE	Emerging RCT evidence for muscle tone, skin firmness, and wrinkle reduction at twelve weeks of consistent use. ^{11,12}
EMS for muscle definition	MODERATE	Body NMES well-evidenced (Cochrane review). ¹³ Facial-specific RCT evidence is smaller but consistent.
Bakuchiol	MODERATE	Comparable twelve-week results to retinol, with less irritation, in a double-blind RCT. ¹⁴

INTERVENTION	EVIDENCE	WHAT IT DOES
Peptides (Matrixyl, copper)	MODERATE	Smaller trials show wrinkle reduction. Useful adjunct, not first-line.
Manual lymphatic drainage, cosmetic	MODERATE	Real reduction in puffiness short-term. Strong evidence for post-surgical edema. Long-term cosmetic claims weaker. ¹⁵
Cold exposure (cryo)	MODERATE	Short-term depuffing via vasoconstriction. No evidence for long-term structural change.
Mediterranean-style diet	MODERATE	Polyphenol intake correlates with reduced photoaging in observational studies.
Gua sha, jade rolling	LIMITED	Mostly anecdotal cosmetic evidence. Useful as a delivery vehicle for the manual work, not as the work itself.
Collagen drinks and supplements	LIMITED	Mixed studies. Effect size in trials is small. Most marketed brands cite the same handful of papers.
Facial yoga and exercise programmes	LIMITED	One small RCT (Alam 2018) showed modest results. Not yet replicated at scale. ¹⁷
Vibrating wands and rollers	LIMITED	No published RCT evidence for facial use. Pleasant sensation is not the same as measurable change.
Plasma pens, microneedling pens (home)	LIMITED	Procedural microneedling has evidence. Home pens at lower needle depth have weak evidence and real infection risk.

The honest read

The strongest interventions are sun protection, retinoids, vitamin C, LED, and sleep, in roughly that order. Microcurrent comes next. Manual work is good for puffiness, weaker for long-term structural change. Anything that puts gua sha and tretinoin in the same sentence as equivalent is not reading the evidence. The five-product shelf in chapter eight will get you ninety per cent of the available result.

IN THE STUDIO

Every strong-evidence category, one workout.

Sculpt Signature is built around the interventions with the most evidence behind them: lymphatic clearance, manual sculpt, microcurrent, EMS in held positions, and recovery. Forty minutes, photographed and assessed against the file at session one.

+44 20 3951 3429 · 10 Portman Square, W1H 6AZ · £110 for forty minutes.

The five-minute morning protocol.

Five minutes covers the four interventions with the strongest evidence behind them. Most of the long-term result comes from doing this consistently for years. The morning routine is shorter on purpose, because the bigger active work happens at night.

0:00 to 1:00

One minute of lymphatic sweep

While the kettle boils. Chapter five covers the technique. The point at this hour is to reduce overnight puffiness around the eyes and lower face before makeup goes on. A salty meal or alcohol the night before makes this minute worth more.

1:00 to 2:00

Gentle cleanse

A pH-balanced gel or cream cleanser. Avoid foaming surfactants that strip the barrier. Pat dry, do not rub. La Roche-Posay Toleriane is a reasonable starting point if you are unsure. Lukewarm water, never hot.

2:00 to 3:00

Vitamin C serum

L-ascorbic acid 10 to 20 per cent, applied to dry skin. Wait sixty seconds before the next layer. Most of vitamin C's anti-ageing work happens in daylight, paired with sunscreen, by neutralising free radicals from UV.⁷ If your skin tolerates it, this is a non-negotiable morning step.

3:00 to 4:00

Moisturiser

Pick one. The active is not the moisturiser. The moisturiser holds the barrier. CeraVe AM Facial Moisturising Lotion or La Roche-Posay Toleriane Sensitive Fluide are both sound. If your skin is dry, look for ceramides. If oily, gel.

4:00 to 5:00

Broad-spectrum sunscreen, SPF 30 or higher

The most evidence-backed anti-ageing step in any routine.⁴ Reapply if outdoors past the early hours. La Roche-Posay Anthelios and Paula's Choice Resist are dependable. You need a teaspoon of product to cover the face and neck. Half that dose gives roughly half the protection.

Layering rules

- Thinnest to thickest. Serum before cream before sunscreen.

- Wait sixty seconds between layers if any of them is active.
- Sunscreen is always the last step before makeup.
- Use enough sunscreen. Half a teaspoon for the face, the same for the neck.

Common morning mistakes

- Skipping vitamin C because the skin "feels fine". Vitamin C is preventative, not corrective.
- Layering too many actives. Vitamin C plus retinoid plus AHA in the morning is too much.
- Using last summer's sunscreen. The active filters degrade. Replace annually.
- Forgetting the neck and the back of the hands. The face will out-age them.

FROM THE WELLNESS BOUTIQUE

Morning routine essentials

The Wellness boutique stocks La Roche-Posay, CeraVe, Paula's Choice and Medik8 alongside the supplement range. Pick one cleanser, one vitamin C, one moisturiser, one sunscreen. Replace vitamin C every three months once opened, sunscreen annually.

thewellnesslondon.com

The fifteen-minute evening protocol.

The evening is where the bulk of the active work happens. This is the routine the studio recommends to clients between fortnightly sessions. The first five minutes (manual sculpt) is the one that visibly changes the face most quickly.

0:00 to 2:00 **Double cleanse**

Oil or balm cleanser first to lift sunscreen and sebum. Water-based cleanser second to remove residue. Two minutes total, full face and neck. This step matters more than people think. Incomplete cleansing is the most common reason for irritation when retinoids are introduced.

2:00 to 7:00 **Manual sculpt, five minutes**

Chapter five covers the technique in detail. Slow, deliberate strokes along the lift lines. A facial oil makes the work glide and reduces drag on the skin. This is the single highest-yield five minutes in the routine, and the one most people skip.

7:00 to 12:00 **Microcurrent, five minutes**

If you have a device. Apply the conductive primer the manufacturer specifies. Hold positions on the lift lines, do not just sweep. Chapter six covers what to expect at home versus in the studio.

12:00 to 14:00 **Treatment serum**

Retinoid is the gold standard, three to five evenings a week to start. Build to nightly over six to twelve weeks. If retinoid is not tolerated, bakuchiol is a reasonable alternative with comparable twelve-week results.¹⁴ Peptide serum on the alternate evenings if the budget allows.

14:00 to 15:00 **Recovery cream**

Ceramide-rich, fragrance-free. The point is to seal the barrier and let the actives work overnight without irritation. CeraVe PM is reasonable. Medik8 Advanced Night Restore is a step up.

The retinoid build-up protocol

1. Weeks one and two: lowest strength (Crystal Retinal 1, retinol 0.025 per cent), three evenings per week, alternating.

2. Weeks three to six: same strength, four to five evenings per week.
3. Week seven onwards: nightly at the same strength, or step up to the next level.
4. Step up only when nightly is fully tolerated for two consecutive weeks.

If you are getting irritation

- Use the sandwich method: moisturiser, then retinoid, then moisturiser again.
- Skip retinoid for two evenings, then resume at lower frequency.
- Drop AHA or BHA from the routine entirely while introducing retinoid.
- Cold compresses can help with redness during week one to four.

FROM THE WELLNESS BOUTIQUE

Evening routine essentials

Medik8 Crystal Retinal at the strength your skin tolerates (1, 3, 6, 10, 20 , start low). NIOD CAIS for the targeted antioxidant work. La Roche-Posay Toleriane to recover on retinoid evenings.

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IN THE STUDIO

Where home stops, the studio starts.

The home protocol holds the result. The forty-minute Sculpt Signature builds it. For documented progress, the four-session Sculpt Course runs fortnightly across eight weeks with photographs at session one and four.

+44 20 3951 3429 · 10 Portman Square, W1H 6AZ · £440 for the four-session course.

The lymphatic sculpt, the core technique.

The single most useful technique you can learn for the face. The evidence for cosmetic benefit is moderate, strong for puffiness reduction and post-procedure swelling, weaker for long-term structural change.¹⁵ The puffiness reduction is real, immediate, and free.

What the lymphatic system does

A network of vessels and nodes that drains tissue fluid back into circulation. The face has limited lymphatic capacity, which is why a salty meal, a poor night's sleep, or an alcoholic evening shows up immediately under the eyes and along the jaw. Manual drainage moves fluid toward the lymph nodes that can clear it. Primarily the cervical chain along the side of the neck, the supraclavicular nodes above the collarbone, and the parotid nodes in front of the ear.

The seven-step technique

1. **Open the gates.** Pump the supraclavicular nodes above the collarbone with light, slow pressure. Fifteen pumps each side. Fluid flows downhill, the gates need to be open before you push fluid toward them.
2. **Side neck, top to bottom.** With flat hands, sweep from the corner of the jaw down the side of the neck to the collarbone. Five to ten passes each side. Light pressure only. Lymphatic vessels sit just under the skin and collapse under firm pressure.
3. **Under the jaw.** Knuckle along the underside of the jaw line from chin out toward the ear. Five passes each side. The submandibular nodes sit here. They are the next staging post on the way to the cervical chain.
4. **Lower face, jaw to ear.** Use the flat of two fingers, sweep from the corner of the mouth across the cheek to the ear. Five passes each side. The parotid nodes in front of the ear are where you are draining toward.
5. **Mid face, nose to temple.** From the side of the nose, sweep up and outward to the temple. Five passes each side.
6. **Brow and forehead.** Centre out to the temples, then down the side of the face to the ear. Five passes.
7. **Drain back down.** One last sweep from the temples down the side of the neck to the collarbone, both sides. The fluid you have moved exits here.

The morning two-minute version

If you do not have time for the full sequence, do steps one, two, four and seven. That is the minimum that opens the gates and clears the lower face, which is where most overnight puffiness sits.

Common mistakes

- **Pressing too hard.** Lymphatic vessels collapse under pressure. Light is correct. If your fingertips blanch the skin, you are too firm.
- **Skipping the neck.** The neck is where the fluid exits. If you only work the face, you are pushing fluid into vessels that are not open.
- **Going too fast.** The pace is slow. Each stroke is two to three seconds.
- **Stopping at week two.** The result on day one is real but small. The result over thirty days is what changes how the face looks at rest.

HANDS OR TOOLS

Hands are the foundation. A gua sha or sculpt ball can extend the work along the jaw and over the masseter where you cannot apply pressure with the fingertips alone. The Wellness boutique stocks the bian stone gua sha and the stainless contour ball the studio uses.

thewellnesslondon.com

IN THE STUDIO

A trained pair of hands runs deeper.

The hands version of the studio protocol is Sculpt Hands, forty minutes, no electrical work, pregnancy-safe from the second trimester. The depth a trained therapist reaches is the difference between five minutes at home and a full result.

+44 20 3951 3429 · 10 Portman Square, W1H 6AZ · £110 for forty minutes.

Microcurrent at home.

Microcurrent is the most evidence-backed home tool for the muscle layer. The evidence base is younger than retinoids but it is real. Twelve weeks of consistent use, five times per week, is the bar for measurable change.

What the evidence says

A 2019 double-blind RCT by Hsieh and Chang specifically investigated microcurrent on facial appearance and found measurable changes in perceived facial ageing.¹¹ A separate RCT of facial neuromuscular electrical stimulation showed significantly higher facial radiance, skin tone, and reduced wrinkles after twelve weeks of treatment, five times per week, against a control group.¹² A 2025 narrative review confirmed the cellular mechanisms. Microcurrent influences ATP production, blood flow and oxygenation, protein synthesis, and waste removal at the cellular level.¹⁸

The honest read: the effect size is moderate, the time-to-result is twelve weeks at minimum, and consistency matters more than intensity. Three sessions a week for six weeks will not move the needle. Five sessions a week for twelve weeks will.

What to look for in a device

- **Sub-sensory output, under 1 mA.** If you feel it strongly, it is too strong. The evidence base is built on currents in the microampere range.
- **Two-prong delivery.** Allows you to bracket a muscle group rather than just slide along the surface.
- **Conductive primer.** Microcurrent will not pass through dry skin properly. Use the primer the device specifies, not a substitute.
- **Multiple intensity levels.** Start at the lowest. Build over weeks, not within a session.
- **Sealed, washable contacts.** Hygiene matters when you are running this five evenings a week.

How to use it

Apply primer to clean skin. Hold the prongs at the start of a lift line. For example, at the corner of the mouth. Trace slowly to the temple, holding for two to three seconds in three to four positions along the way. Do each lift line three times. Five lift lines on each side of the face. Five minutes total. Five evenings per week. The first measurable change typically appears at week six. The full home result appears at week twelve.

Pairing with retinoid

Microcurrent before retinoid, not after. The conductive primer rinses off, then the retinoid goes on cleanly. Reverse the order and the primer interferes with retinoid absorption.

The home ceiling

Home microcurrent works best as maintenance. Studio devices run higher intensity, the protocols are sequenced to the day's findings, and a trained operator can hold positions you cannot reach yourself. A reasonable model is to alternate. Monthly studio session for the depth, daily home device for the consistency.

FROM THE WELLNESS BOUTIQUE

The studio's microcurrent recommendation.

Two-prong, sub-sensory, four levels, with the conductive primer the studio uses. Replacement primer ships every three months on subscription.

thewellnesslondon.com

IN THE STUDIO

EMS Lift Intensive, the studio version.

For clients already training the muscle layer at home, EMS Lift Intensive is the fifty-minute studio session running a full programme across cheek, jaw, and brow.

+44 20 3951 3429 · 10 Portman Square, W1H 6AZ · £150 for fifty minutes.

Light therapy, wavelengths and time.

LED is the second most evidence-backed home tool, after retinoids. Wavelength matters. Irradiance matters. Time matters. Cheap masks work if the wavelengths and irradiance are correct. Expensive masks do not work if they are not.

What the evidence says

A 2007 split-face RCT by Lee and colleagues compared 633 nm red light, 830 nm near-infrared, the combination, and a sham. Twice per week for four weeks. All three active arms showed statistically significant wrinkle reduction. 26 per cent, 33 per cent, and 36 per cent respectively, plus 19 per cent skin elasticity improvement.⁸ A 2014 RCT by Wunsch and Matuschka confirmed increased intradermal collagen density on ultrasonography after thirty sessions.⁹ A study by Goldberg and colleagues reported softening of periorbital wrinkles in eighty per cent of subjects with combined 633 nm and 830 nm treatment.¹⁹

Wavelengths and what each does

- **633 nm red.** Penetrates to the mid-dermis. Supports collagen synthesis and reduces inflammation. Most evidenced wavelength for anti-ageing.
- **830 nm near-infrared.** Penetrates deeper. Supports tissue repair, elasticity, and recovery from inflammation.
- **415 nm blue.** For acne, antibacterial action against *P. acnes*. Not anti-ageing.
- **590 nm amber.** Some collagen evidence, less than red. Optional.

For anti-ageing work, the combination of 633 nm and 830 nm is the most studied. Pick a mask or panel that delivers both. The manufacturer should publish irradiance figures (in mW/cm²) at the skin surface. If they do not, the device is not credible.

How to use it

Three to five sessions per week, ten minutes per session, on clean skin without sunscreen or active serums. Eyes closed, or use the eye protection the device provides. Best results show at week six to twelve, with continued improvement at thirty sessions.

How to test if your device is actually working

The cheap home masks are a mixed bag. A test that takes a week: photograph the back of your hand standardised (chapter twelve covers the technique). Use the LED on it daily for ten minutes. By day seven, you should see a small but visible change in skin tone and texture. If the back of the hand does not respond at all, the device is probably not delivering the wavelengths or irradiance the box claims.

Combining with retinoid

LED in the morning or early evening, before the retinoid step. Not after. The LED session ends with a clean, dry face that is ready for the evening protocol.

FROM THE WELLNESS BOUTIQUE

LED at home.

The Wellness boutique stocks LED masks meeting the 633 plus 830 specification, with published irradiance figures. The boutique team can advise on the difference between the panel and the mask formats.

thewellnesslondon.com

IN THE STUDIO

LED Recovery booster.

Stack the studio-grade panel onto any workout, fifteen minutes, hands-free. Most often paired with Sculpt Signature for the recovery phase.

+44 20 3951 3429 · 10 Portman Square, W1H 6AZ · £40 for fifteen minutes.

Topicals that move the needle.

Five topicals carry most of the evidence for changing how the skin looks over time. Everything else is supporting cast. If your shelf has more than seven products, you are probably overcomplicating it.

1. Topical retinoid, the gold standard

Tretinoin (retinoic acid) is the most evidence-backed topical in dermatology for photoaging. A 2022 systematic review of randomised controlled trials confirmed efficacy from one month and lasting after twenty-four months.⁵ A 2024 network meta-analysis of 23 RCTs and 3,905 participants placed isotretinoin, retinol, and tretinoin as the most effective interventions for fine wrinkles.⁶

Protocol. Start at the lowest strength, three evenings per week. Build to nightly over six to twelve weeks. Wait twenty minutes after cleansing for skin to dry. Applying retinoid to damp skin increases irritation. Apply a pea-sized amount to the full face. Always pair with morning sunscreen, since retinoids increase photosensitivity. Expect a four to six week adjustment period of dryness and mild flaking. This is the skin acclimating, not damage.

If retinoid is not tolerated, bakuchiol is the best-evidenced alternative. The 2019 BJD double-blind RCT found comparable twelve-week results with significantly less irritation.¹⁴

2. Vitamin C, antioxidant and collagen support

L-ascorbic acid 10 to 20 per cent, applied to dry skin in the morning. Vitamin C does its work in daylight by neutralising UV-generated free radicals and supporting collagen synthesis.⁷ The molecule is unstable. Buy small bottles, store in dark glass, replace when the colour shifts to amber.

3. Niacinamide, barrier and pigment

5 per cent niacinamide improves the skin barrier, reduces redness, evens pigment, and is well-tolerated alongside retinoids.¹⁰ Useful as a single morning serum, or as a moisturiser ingredient.

4. Sunscreen, the most evidence-backed of all

Broad-spectrum SPF 30 minimum, applied every morning, reapplied if outdoors past the early hours. The evidence base for sunscreen as anti-ageing is larger than the evidence for any other intervention in this guide. La Roche-Posay Anthelios, Paula's Choice Resist, and Beauty of Joseon are dependable. Mineral or chemical works. The one you actually wear is the right one. Application dose matters. A teaspoon for the face and neck. Half that, half the protection.

5. Peptides, the supporting cast

Matrixyl 3000, copper peptides, and palmitoyl-tripeptide-1 are the most studied. Smaller trials show wrinkle reduction. Useful as a supplement to retinoid and vitamin C, not a replacement.

What not to combine

- **Retinoid plus AHA** (glycolic, lactic) in the same routine. Too much exfoliation, barrier damage. Alternate evenings.
- **Vitamin C plus niacinamide**. Historically debated. Modern formulations tolerate the combination, but if your skin is sensitive, separate them by morning and evening.
- **Retinoid plus benzoyl peroxide**. BPO neutralises retinoid. Use BPO in the morning if you need it for acne, retinoid at night.
- **More than two actives** in a single routine. The actives compete for absorption and the chance of irritation rises sharply.

The five-product shelf, in order of priority

1. Broad-spectrum SPF, every morning.
2. Retinoid (or bakuchiol if not tolerated), every evening once acclimated.
3. Gentle cleanser, twice daily.
4. Ceramide-rich moisturiser, evening.
5. Vitamin C, morning, on dry skin.

That shelf delivers ninety per cent of the achievable result. The other ten per cent is microcurrent, LED, sleep, and discipline.

FROM THE WELLNESS BOUTIQUE

The five-product shelf.

Medik8 Crystal Retinal at the strength your skin tolerates. Paula's Choice C15 Booster for vitamin C. NIOD or The Ordinary niacinamide. La Roche-Posay Anthelios for sunscreen. CeraVe PM for the recovery layer. The boutique is on Portman Square. Online ordering ships next day across the UK.

thewellnesslondon.com

Sleep, stress, and the face.

A study of sleep quality and skin found that poor sleepers showed measurable signs of accelerated intrinsic ageing, including fine lines, uneven pigment, and slower barrier recovery, at the same chronological age as good sleepers.¹⁶ No serum has the effect size of seven hours of sleep.

Sleep duration

The evidence converges on seven to nine hours for adults. Below six is associated with measurable skin changes inside a week of disrupted sleep. Sleep quality matters as much as duration. The deepest restorative phases happen in the first half of the night, and alcohol within three hours of bed cuts that phase short.

Sleep position

Side and stomach sleeping creates compression lines on the face that, repeated over decades, become permanent. Supine (on the back) is best for the face. A silk pillowcase reduces friction. The cosmetic case for changing your sleep position is thin if you sleep well, more significant if you do not. Do not sacrifice good sleep for sleep position.

Stress and cortisol

Chronic raised cortisol degrades collagen, suppresses skin-cell turnover, and impairs barrier repair. The mechanism is well-described in the literature on glucocorticoid effects on skin. The interventions with most evidence are sleep, daily exposure to natural light in the morning, regular exercise, and reducing the inputs that drive sustained stress. Skincare cannot out-perform a year of poor sleep.

Mouth breathing and the jaw

Habitual mouth breathing changes facial development in childhood and posture in adulthood. Nasal breathing, particularly during sleep, supports better lower-face position and a sharper jaw line. If you wake with a dry mouth most mornings, this is worth investigating with a clinician. A simple sleep study often identifies the cause and frequently the fix is straightforward (positional changes, weight management, sometimes a CPAP or oral appliance).

OPTIMAL SLEEP DURATION

7 to 9 hours

Below 6 hours linked to faster ageing¹⁶

BEST SLEEP POSITION FOR THE FACE

Supine, on the back

Side and stomach create permanent compression lines

The bedroom audit

- No phone within reach. Charge it across the room.
- Bedroom dark enough to lose your hand at arm's length.
- Cool. 18 degrees Celsius is a reasonable target.
- Caffeine cut-off twelve hours before bed. The half-life is longer than people assume.
- Alcohol three hours before bed at the latest. Earlier is better.

IN THE BUILDING

The Wellness runs sleep clinics, ENT consultations, and private GP through the medical practice on the floors above the FaceSculpt studio. If sleep is the issue and you have not seen anyone about it, the GP intake is the place to start.

thewellnesslondon.com

What you eat shows on your face.

Nutrition is the second-strongest lever after sun protection. The evidence does not support specific foods as miracle interventions, but it supports a small number of clear patterns.

Omega-3 fatty acids

Multiple studies link omega-3 intake (oily fish, flax, algae) to reduced skin inflammation and improved barrier function. A controlled trial by Pilkington and colleagues showed protective effects against UV-induced skin damage with omega-3 supplementation.²⁰ Two to three servings of oily fish per week, or supplementation if dietary intake is low.

Polyphenols

The colour pigments in berries, dark chocolate, green tea, olive oil, and red wine. Antioxidant action that supports skin against oxidative damage. The evidence for diet-derived polyphenols is stronger than for supplement extracts. Extract pills do not replicate the food matrix.

Vitamin C and zinc

Both are required for collagen synthesis. Citrus, peppers, and dark leafy greens for vitamin C. Shellfish, pumpkin seeds, and beef for zinc. Deficiency in either is not subtle on the face. The studio recommends a baseline blood panel for new clients with persistent skin issues. The Wellness medical team runs these on the floor above.

Glycation, the sugar problem

Sustained high blood sugar produces advanced glycation end-products (AGEs) that cross-link collagen fibres and stiffen the skin. The evidence for this mechanism is strong. The intervention is dietary. Reducing refined sugars and ultra-processed foods has skin benefits over years, not weeks. The rule of thumb in the studio is simple. If a packet has more than five ingredients, none of which you would have in your kitchen on its own, treat it as occasional.

Alcohol

Dehydrates, dilates capillaries, disrupts sleep, and contributes to glycation. The face shows alcohol consumption faster than the body does. Reduction is the lever. Abstinence is not required for skin benefit, but moderation produces visible change inside three months in most adults. Two alcohol-free days per week is a useful starting target.

Hydration

The link between drinking-water intake and skin appearance is weaker in the literature than wellness culture suggests. Severe dehydration shows on the face. Beyond avoiding that, the marginal effect of moving from two to three litres per day is small. Hydration is not the same as moisturisation.

The Mediterranean baseline

If you ignore everything else above and just default to a Mediterranean-style diet (vegetables, olive oil, oily fish, legumes, whole grains, low-to-moderate red meat, occasional wine, very little ultra-processed), you cover the omega-3, polyphenol and glycation patterns in one move.

FROM THE WELLNESS BOUTIQUE

Supplement support.

The Wellness supplement range covers algae omega-3, zinc picolinate, iron bisglycinate, and probiotics. Formulated for absorption and dosed at the levels the medical team uses with patients. Clinical-grade rather than retail-grade.

thewellnesslondon.com

What home cannot do.

Most of this guide is what you can do at home, alone, in fifteen minutes a day. This chapter is honest about where the home protocol ends, and why a studio session is the next layer rather than an indulgence.

Depth of manual work

The depth a trained pair of hands reaches on the masseter, the platysma, the sub-mandibular line, and the cervical chain is not replicable on yourself. Anatomically, you cannot reach your own deep neck. Practically, you cannot apply the pressure required without recruiting your own neck and shoulder muscles to compensate. A forty-minute studio workout covers ground a five-minute home routine cannot.

Equipment ceiling

Studio microcurrent runs at higher intensity than home devices, on protocols sequenced to the day's findings. Studio EMS reaches muscle groups home devices cannot reliably stimulate. Studio LED panels deliver irradiance levels that get you to the result in four weeks rather than twelve.

Photographic record

You cannot take a standardised photograph of yourself. The lighting is not consistent, the angle is not consistent, the expression is not consistent, the time of day is not consistent. The change is decided against the file, not against impression, which means the file has to be standardised. Chapter twelve covers what is possible at home. The studio version is more rigorous and more useful.

The trained eye

A therapist who has worked on hundreds of faces sees what you cannot see in your own. The asymmetries you carry from chewing on one side, sleeping on one side, holding tension in one shoulder. These show up in the assessment, and the work is sequenced around them. Your daily mirror does not show this.

Discipline without scheduling

Honestly, this is the biggest one. The home protocol works if you do it. The studio session happens whether you remember to or not. Booking the appointment is the discipline.

Showing up is the routine. Most people fall off the home protocol at week three. Members do not fall off, because the next visit is in the diary.

The home protocol holds the result. The studio builds it.

THREE COMMITMENTS AT THE STUDIO

Pick the depth that fits.

Sculpt Day for a single intensive visit at £495. Sculpt Course for the eight-week protocol at £440, the most chosen. Sculpt Year for the year-long photographic record at £2,400.

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How to measure progress at home.

Most people doing skincare cannot tell whether anything is working. This is not because the products do not work. It is because the human eye, looking at its own face every morning, is the worst possible instrument for noticing slow change. This chapter teaches a simple skill that fixes that.

Why daily mirrors lie

You see your face every morning under different light, after different sleep, at different times of day, with different recent meals. Each variable shifts how the face looks by more than three months of retinoid use will. Without a fixed reference, you cannot detect the signal under all that noise. Your perception drifts gradually with the change, so you stop seeing improvement even when it is happening.

The standardised photograph

The fix is the same fix the studio uses: take photographs under controlled conditions. Five rules.

1. **Same light.** Same window, same time of day, same direction. Morning light from a north-facing window is the most reliable in the UK. Avoid direct sunlight, avoid overhead bathroom light. If you can only use indoor light, the same kitchen ceiling pendant every time.
2. **Same angle.** Phone at eye level, arm extended, the same distance every time. Mark the spot on the floor where you stand. Mark the wall the phone faces.
3. **Same expression.** Relaxed face, mouth closed but not pressed, eyes neutral. No smile, no pout. A consistent neutral. Practise five times before the first photograph.
4. **Same background.** A plain wall. The same plain wall every time. Cream or off-white shows the face most accurately.
5. **Three angles, not one.** Front, left side at forty-five degrees, right side at forty-five degrees. Change happens asymmetrically. One side will respond before the other, and you will only see this if you photograph both.

What to look for

- **Lift line on the side angle.** The line from the corner of the mouth to the ear. This is where Sculpt Signature is built around. Measurable change here is the clearest signal.
- **Cheek volume.** Compare the apple of the cheek between weeks. Most home work does not change this materially. Fat compartment work is procedural. If it does change, it is because lymphatic clearance has reduced chronic puffiness.
- **Under-eye.** Fine lines and pigment over weeks. Tretinoin shows here first.
- **Texture.** Pore visibility, smoothness, evenness. Vitamin C and retinoid show here over twelve weeks.
- **Pigment.** Sun spots, post-inflammatory marks. The slowest of all to change. Six months minimum.

What not to look for

- **Day-to-day puffiness.** Fluctuates with sleep, salt, alcohol, hormones, the weather. Not a signal.
- **One-off bad photos.** A single photograph at the wrong moment is noise. Trust the trend across four photographs at four-week intervals, not any single one.
- **Skin you saw on someone else's Instagram.** Filters, lighting, post-production, surgery. Compare yourself to your own week-zero photograph. That is the only fair comparison.

Cadence

Take photographs at week zero (today), week four, week eight, week twelve, and then quarterly thereafter. Save them in a dated folder. Compare the latest three months against the same month last year. The skin changes you see at the year mark are the changes that matter.

The honest baseline

Most people are unhappy with their week-zero photograph. That is normal. The week-zero photograph is not for impressing anyone. It is the only fixed reference you have, and twelve months from now it is the document that tells you whether any of this worked.

IN THE STUDIO

The studio takes the photographs for you.

Standardised photographs at session one and session four of the Sculpt Course. Annual review for members. AI-guided assessment captures six symmetry markers, three muscular-tension areas, and two skin-condition scores. The full record is yours, available on request inside two working days.

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The evidence cited.

Where this guide states a clinical claim, the underlying study is listed below. The studio uses this same reference list when patients ask, and updates it as new trials are published.

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This guide is intended as general advice. It does not constitute medical advice and is not a substitute for clinical assessment. If you have a skin condition that concerns you, book a consultation with a clinician. The Wellness medical practice on the floors above the FaceSculpt studio offers private GP and dermatology consultations. +44 20 3951 3429 or team@thewellnesslondon.com.